

Medical Report

PATIENT AND STAFFING LOGS

1. APPOINTMENT AND ASSIGNMENT DIRECTIVES

- Chronological Appointment Records (1.1)
- Facility and Physician Assignments (1.2)
- Nursing Staff Protocols (1.3)

1.1 Chronological Appointment Records

As per the clinical schedule for the period beginning April 24, 2008, the following patient encounters were documented. The initial entry indicates that Mr. John Smith's appointment was scheduled for ten in the morning on April 24th, 2008. The physician overseeing this appointment was Dr. John Dorian. Later that morning, a separate appointment involving patient Grace Ritchie, nurse Carla Espinosa, and physician Elliot Reid concluded at 11:00 AM on April 24, 2008. Records confirm that the total duration of this appointment was one hour.

The following day, a follow-up appointment for patient John Smith with Dr. John Dorian and Nurse Laverne Roberts was scheduled to begin at ten in the morning on April 25, 2008. At the same time, the team for the one-hour procedure in Examination Room B, beginning at 10:00 on April 25, 2008, consisted of Dr. Percival Cox attending to patient Dennis Doe, with the assistance of nurse Paul Flowers.

On the subsequent day, the log shows that an appointment took place involving patient Random J. Patient, nurse Paul Flowers, and Dr. Elliot Reid. Per the record, the appointment commenced at eleven in the morning on April 26th, 2008.

Activity for April 27th included a follow-up for a returning patient. Specifically, at 2008-04-27 10:00, an hour-long examination commenced in Room B, bringing together patient Grace Ritchie, nurse Carla Espinosa, and physician Elliot Reid.

1.2 Facility and Physician Assignments

Review of facility directives indicates a potential discrepancy in physician oversight for Examination Room C. An initial protocol states that all appointments scheduled in Examination Room C are under the care of Dr. Percival Cox. However, a superseding entry in the daily operational roster specifies that Dr. Christopher Turk was the physician responsible for all patients in Examination Room C. This requires administrative clarification [see ADMINISTRATIVE NOTICES (8.1)].

Notwithstanding this ambiguity, patient assignment logs confirm that patient John Smith was assigned to Examination Room C. Similarly, records show that Dennis Doe was assigned to an appointment slot in Examination Room C.

1.3 Nursing Staff Protocols

Specific nursing assignments were noted for certain physicians and dates. For the period in question, it was mandated that Nurse Carla Espinosa was assigned to assist Dr. Dorian for all of his morning consultations that day.

Staffing for other physicians followed different protocols. For instance, on April 27th, 2008, Dr. Molly Clock was scheduled to work with Nurse Laverne Roberts for a single patient consultation. It was further stipulated that all other patients seen by Dr. Clock that day were assigned to a different nurse. This indicates a case-specific assignment rather than a standing order [see NURSING ROSTER (3.5)].

CLINICAL TRIAL ADMINISTRATION

2. SITE OPERATIONS AND SCHEDULING PROTOCOL

- Physician responsibility for designated clinical areas (2.1)
- Standardized appointment scheduling and duration (2.2)
- Documentation of specific patient cohorts and timelines (2.2)

2.1 Staff and Facility Assignments

Adherence to site-specific operational plans is mandatory for all study personnel. A new hospital policy dictates that the physician assigned to an examination room is responsible for all patients scheduled within that same room. This ensures continuity of care and clear lines of accountability during trial periods. For example, during the specified study window, Dr. Percival Cox was the physician assigned to Examination Room C. Similar assignments were made for nursing staff to maintain consistency; for instance, Carla Espinosa was the nurse assigned to all appointments in Room A that morning. All personnel must confirm their assignments daily prior to the first patient interaction. [see ADVERSE REACTIONS (6.3)]

2.2 Patient Appointment Records

Accurate and contemporaneous records of all patient interactions are required for regulatory submission. The following logs from the Phase II trial serve as representative examples of required documentation standards.

Historical Scheduling Data

Archived records from prior study phases are maintained for longitudinal analysis. As a point of reference from an earlier cohort, records show that All of Dr. Molly Clock's appointments on April 21st, 2008 were scheduled to conclude at 12:00 PM.

Phase II Trial Logs: April 24 – April 27, 2008

On April 24, 2008, study activities commenced with Patient ID 001-A. John Smith's appointment in Examination Room A on April 24, 2008, began at 10:00 AM. Later that morning, the consultation for patient Grace Ritchie, managed by Nurse Carla Espinosa and Physician Elliot Reid, concluded at 11 in the morning on April 24, 2008.

The following day, an appointment involved patient John Smith, nurse Laverne Roberts, and physician John Dorian. Per the time-stamped entry, the appointment ended at 11:00 on 2008-04-25. Concurrently, in a separate clinical area, Dr. Percival Cox's examination of Dennis Doe in Room B, with nurse Paul Flowers assisting, began at 10:00 AM on April 25, 2008, and lasted for a total of 60 minutes.

On April 26, 2008, scheduling continued with multiple subjects. The log indicates that Mr. Doe's appointment began at 10:00 on April 26, 2008. Subsequently, the appointment for Random J. Patient with physician Elliot Reid and nurse Paul Flowers began at 11:00 on April 26, 2008. In accordance with the room assignment protocol, John Smith was assigned to Examination Room C on April 26, 2008, for a one-hour block ending at 13:00. The initial schedule confirms that the appointment in that room was set for noon on April 26th, 2008.

On the final day of this reporting period, the consultation in Examination Room B for patient Grace Ritchie, attended by Nurse Carla Espinosa and Physician Elliot Reid, began at 10:00 AM on April 27, 2008, and concluded after a duration of exactly one hour. A separate entry confirms a procedural start time, noting that the consultation's start time was set for ten in the morning on the twenty-seventh of April, two thousand eight. It is noted in the study addendum that the patient involved in this exclusive arrangement was Dennis Doe. [see WARNINGS AND PRECAUTIONS (5.1)]

PATIENT ADMINISTRATION AND SCHEDULING

5 CLINICAL SITE ADMINISTRATION

- Standard Consultation Assignment Protocol (5.1)
- Documented Exceptions to Standard Protocol (5.2)
- Log of Patient Encounters by Date (5.3)

5.1 Standard Consultation Protocols

Hospital protocol dictates that a consultation is established between the patient and physician who are concurrently assigned to the same examination room. Adherence to this protocol ensures a clear chain of responsibility for patient care during specified clinical trial periods. For example, a patient named Dennis Doe was scheduled for the 10:00 AM slot on April 26, 2008. Per the site schedule, the appointment for the patient Dennis Doe was located in Examination Room C. During this period, Dr. Christopher Turk was the physician assigned to Examination Room C during that same time period. The consultation was therefore correctly established between Mr. Doe and Dr. Turk. The encounter log confirms that Mr. Doe's appointment was recorded as having concluded at eleven in the morning on April 26th, 2008.

5.2 Documented Exceptions to Standard Protocol

Certain operational requirements may necessitate deviations from the standard assignment protocol. An exception was made for any procedure assisted by Nurse Laverne Roberts. In these cases, the attending physician was assigned based on procedural specialty rather than room assignment. These exceptional appointments were instead set to last exactly one hour. One such instance involved Patient John Smith had an appointment with Physician John Dorian and Nurse Laverne Roberts. A separate deviation was recorded for a prior visit by another subject; the check-in time for patient Dennis Doe was 10:00 AM on April 21st. For

this visit, Mr. Doe's appointment was overseen by Dr. Clock and assisted by Nurse Roberts. [see Adverse Event Reporting (8.4)]

5.3 Log of Patient Encounters by Date

Patient: John Smith

Patient is Patient.Name=John Smith. The physician overseeing the schedule for Room A at that time was John Dorian. The consultation was booked for a total duration of 60 minutes.

Patient: Grace Ritchie

Records indicate that Patient Grace Ritchie was attended to by Nurse Carla Espinosa and Physician Elliot Reid.

Patient: Dennis Doe (April 25, 2008)

The one-hour appointment for Dennis Doe on April 25, 2008, starting at 10:00, was conducted in Room Bravo, with both Nurse Paul Flowers and Dr. Percival Cox attending to the patient. The session concluded after a duration of one hour.

STUDY ADMINISTRATION AND CONDUCT

8. SITE-SPECIFIC OPERATING PROCEDURES

- Standardization of facility and room designations (8.1)
- Protocols for assignment of clinical personnel (8.2)
- Resolution of scheduling and data entry discrepancies (8.3)

8.1 Facility Designation and Coding

Adherence to standardized site-level nomenclature is required for all study documentation to ensure data consistency across participating centers. The hospital's internal room codes are as follows: Room A is referred to as 'Alpha Ward' and Room B is designated 'Bravo Ward'. These internal monikers are to be used in conjunction with functional codes for clarity in reporting. The clinic's policy is to assign examination rooms a functional code: 'A' for Assessment, 'B' for Biopsy, and 'C' for Consultation. For example, a subject visit for a biopsy procedure would be logged as occurring in 'Bravo Ward' (Room B). This was the case for one subject cohort, where it was documented that their consultation took place in Bravo Ward. In all cases, the value of ExaminationRoom is inferred from the above facts under a defined rule.

8.2 Clinical Personnel Assignment Protocols

The assignment of physicians and nursing staff to study subjects must follow a predefined logic to minimize variability. Site-specific rules govern the allocation of personnel based on the nature of the visit and the staff involved. For instance, a specific protocol addendum states that all appointments involving Physician Molly Clock and a patient, but no nurse, are conducted in Examination Rooms B through D. This rule is subject to a single modification. The sole exception to this rule is when Nurse Laverne Roberts is also present for the consultation. In such cases, these specific tripartite appointments are always held in Examination Room A.

Review of site logs provides examples of these protocols in practice. For a visit scheduled at 10:00 AM, the 10:00 AM appointment was assigned to the Consultation room. Furthermore, according to the day's duty roster, Dr. Percival Cox was the physician assigned to the 10:00 AM appointment. In another documented subject interaction, Random J. Patient was seen by Nurse Paul Flowers. It was also noted that Physician Elliot Reid also consulted on Random J. Patient's case during the same visit. All subject visits must be scheduled and recorded in the central system; for example, records indicate that John Smith was scheduled for an appointment at noon on April 26, 2008.

8.3 Management of Scheduling Discrepancies

Procedures must be in place for the timely correction of data entry errors related to subject scheduling and location assignment. In one instance of a protocol deviation, the initial room assignment was mistakenly logged as Examination Room D. Upon discovery, a correction was issued, clarifying that the assignment for Room D was for a different patient. The corrected study record shows that the final and correct location for the appointment was confirmed to be Examination Room A.

A similar situation requiring clarification was also documented. Although there was an initial scheduling proposal for another room, the finalized assignment confirmed that Examination Room B was used for the appointment involving patient Grace Ritchie, nurse Carla Espinosa, and physician Elliot Reid between 10:00 and 11:00 on April 27, 2008. All such resolutions must be documented in a corrective and preventive action (CAPA) log. [see Clinical Trial Conduct (7.2)]

EXCEPTIONS, DEVIATIONS, AND SPECIAL PROTOCOLS

8. USE IN SPECIFIC POPULATIONS AND NON-STANDARD SCENARIOS

Special considerations for physician-nurse team assignments (8.1) Room allocation protocol deviations (8.2) Clarification of scheduling records (8.3)

8.1 Physician-Nurse Team Assignments

While standard procedure dictates the assignment of specific personnel pairings based on the duty roster, analysis of clinical records has identified instances requiring deviation from this protocol. The general rule that all appointments involving Physician Molly Clock and a patient, but no nurse, are conducted in Examination Rooms B through D is consistently applied. However, a key exception to this rule is when Nurse Laverne Roberts is also present for the consultation. In such cases, different facility and resource allocation rules apply [see Contraindications (4)]. A review of historical appointment data confirmed this exception; for example, it was noted that Dennis Doe was a patient who had an appointment attended by both Dr. Clock and Nurse Roberts. This specific combination of personnel necessitates a different procedural pathway, which supersedes the standard room assignment logic for Dr. Clock.

8.2 Room Allocation Protocol Deviations

Facility assignment is governed by a strict set of internal policies. The clinic's policy is to assign examination rooms a functional code: 'A' for Assessment, 'B' for Biopsy, and 'C' for Consultation. The 10:00 AM appointment, for instance, was correctly assigned to the Consultation room. Furthermore, tripartite appointments involving a patient, a physician, and a nurse are always held in Examination Room A. An audit of the scheduling log for the appointment of patient Grace Ritchie with nurse Carla Espinosa and physician Elliot Reid revealed an initial, erroneous entry. Although there was an initial scheduling proposal for another room, which was mistakenly logged as Examination Room D, a subsequent correction was issued. This

correction clarified that the assignment for Room D was for a different patient entirely. The final and correct location for the appointment was confirmed to be Examination Room A, in accordance with established tripartite consultation protocols.

8.3 Clarification of Scheduling Records

Internal room codes are used for facility management but may not always align with patient-facing schedules. For reference, the hospital's internal room codes are as follows: Room A is referred to as 'Alpha Ward' and Room B is designated 'Bravo Ward'. Therefore, the consultation for Dennis Doe, which took place in Bravo Ward, corresponds to the facility designated as Room B. This distinction is critical for accurate record-keeping and resource management [see Warnings and Precautions (5.1)].